



POLICY

Physical Health Assessment and Monitoring

Scope (Staff):	All Clinical Staff
Scope (Area):	CAMHS

Child Safe Organisation Statement of Commitment

CAHS commits to being a child safe organisation by applying the National Principles for Child Safe Organisations. This is a commitment to a strong culture supported by robust policies and procedures to reduce the likelihood of harm to children and young people.

This document should be read in conjunction with this [disclaimer](#)

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Aim

To provide a standardised approach to regular physical health assessment and monitoring of children attending CAMHS as part of a commitment to improving the physical health outcomes of mental health consumers.

Risk

Poor physical health outcomes for the child relating to:

- Lack of identification of physical health disorders and/or misdiagnosis.
- Poor management of comorbid disorders.

Background

CAMHS applies the [Chief Psychiatrist's Standards for Clinical Care \(CPSCC\)](#), specifically the **Standard: Physical Health Care of Mental Health Consumers**, to improve the physical health outcomes of children who experience mental illness. This policy must be read in conjunction with the CPSCC and the [Mental Health Act \(WA\) 2014](#) (MHA 2014). It should also be read alongside [CAMHS Consent Guideline](#) and [CAMHS Clinical Assessment Policy](#).

This Policy does not cover physical health assessment and monitoring for children who are prescribed psychotropic medication. See the [CAMHS Psychotropic medication – monitoring adverse physical health effects policy](#).

Definitions

Assessment: Process by which the characteristics and needs of a child and their family or carer are evaluated and determined so that they can be addressed. The assessment forms the basis of treatment, care and recovery planning ([CPSCC](#)).

Child: an infant, child, adolescent, or young person under the age of 18 years.

Consent: an agreement from the child and their parent(s)/guardian/s based on an understanding of the implications of a particular activity or decision, and the likely consequences for the child and family.

Family: any parent, legal guardian or identified carer of the child. For a child of Aboriginal or Torres Strait Islander descent, this may be a person from their kinship group who is recognised under customary laws and traditions as their carer (see the definition of 'close family member' under s. 281 of the MHA 2014).

Growth Charts: diagnostic tools based on the [WHO Child Growth Standards](#) used to monitor and assess the nutritional status of infants and young children worldwide. Growth charts include growth indicators such as weight-for-age, length/height-for-age, weight-for-length/height, and Body Mass Index (BMI).

Physical Examination: assessment of the child's physical condition by a medical practitioner; based upon the child's presentation, previous and proposed treatments, and physical health issues common to children with mental illness. It is often an ongoing process and may involve continuing physical examinations based upon test results.

Principles

- Children with mental illness have the right to physical (including dental) health care that is equal to the healthcare provided to the general population.
- All staff have a responsibility for ensuring consumers are supported to obtain equal access to physical health care within their scope of practice (e.g. assistance with accessing no/low fee health care and transport services).
- CAMHS services will actively engage with all services involved in the health and wellbeing of children to enable effective communication and provision of comprehensive care. Services include primary, specialist and other external providers, such as paediatricians, dieticians, dentists, Child Development / Child Health Services, and schools.
- CAMHS will ensure that physical health care needs are considered in the planning and provision of any mental health interventions and are coordinated with equal priority to support recovery.
- Physical health issues may be a consequence of co-occurring health issues, mental illness or side-effects of mental health interventions and treatment. CAMHS has a responsibility for ensuring that the physical health care needs of children accessing their services are identified, assessed and managed appropriately in collaboration with primary, specialist and other health service providers.
- All relevant information regarding a child's physical health care needs must be communicated accurately and in a timely manner to other health care providers, as necessary, and in accordance with confidentiality requirements.
- The multidisciplinary team (MDT) is responsible for the provision and monitoring of health care needs of children within CAMHS. Different members of the MDT will have different roles in this respect.
- All CAMHS clinicians should be able to conduct an enquiry regarding physical health care. This can be achieved through adequate training of clinicians in identifying physical health care needs and informing other multidisciplinary team members on matters such as adverse effects of medication.
- CAMHS clinicians should be aware of services which address children's physical health needs and should work in collaboration with such services.
- CAMHS clinicians must ensure that the physical wellbeing of all children under their care is regularly assessed, and that children and their family's are offered information regarding relevant supports and appropriately referred to other services for further care, as required.

- CAMHS recognises the family's role and facilitates their involvement in the assessment and management of physical health care needs of their children. This includes actively promoting and engaging in 2-way communication.
- Comorbid use of substances, use of non-prescribed medications and lifestyle choices will be recognised in treatment planning.
- CAMHS clinicians who prescribe psychotropic medications are responsible for relevant physical health assessments and monitoring. See the [CAMHS Psychotropic medication – monitoring adverse physical health effects policy](#).

Consent

- Consent to provide physical health care must be established and recorded in webPSOLIS in accordance with the [CAMHS Consent Guideline](#).
- The clinician must clearly explain the nature and purpose of the physical examination and cannot compel the child to have a physical examination. The clinician must make it clear that the child can say no to any part of the assessment, or delay any part of the assessment at any time
- Where a child is reluctant to consent to assessment, consider:
 - Exploring why consent has not been given;
 - Using collaborative ways to reach a decision;
 - Negotiating what part of the assessment is most important at this time and what might be deferred to later;
 - Who might be the best person to communicate with the child about the assessment, and to perform the assessment itself. Consider the gender of the clinician (don't assume that the same gender is always preferred), and which clinician has the best rapport with the child.
 - Offering flexibility around timing of the assessment/observation/monitoring
 - Consider using phrases such as 'have you thought more about when is a good time to do the physical health assessment?'
 - In an inpatient setting, consider asking at meal time or medication time.
- Where a child declines assessment of their physical health care needs despite appropriate advice, the clinician must consider ongoing strategies to assist the child and family to manage their future physical health care needs. Where refusal for a physical examination is considered by the clinician to be a symptom of mental illness requiring treatment, it may be appropriate that the child be assessed by a psychiatrist to determine whether the criterion for involuntary referral is met (MHA s. 241 and Clinicians' Practice Guide to the MHA 2014¹ (CPG) s. 1.9.10).

- Children and their families must be provided with information on the impact of medications on physical health as outlined in the [CAMHS Psychotropic Medication – Monitoring Adverse Physical Health Effects policy](#).
- Prior to coordinating physical health care with external health care providers, a discussion with the child and their family must occur regarding the nature and purpose of the exchange of information and written consent gained, as per the [CAMHS Consent Guideline](#).

Consent – Involuntary Mental Health Patients or Mentally Impaired Accused (MIA) patients

- Informed consent should be sought, as best practice, from involuntary or MIA patients in accordance with the [CAMHS Consent Guideline](#).
 - However, consent is not required for a physical examination or for the taking of samples of blood, saliva, tissue and excreta (MHA s. 241(3)).
- If an involuntary patient is adamant that they will not cooperate with a physical examination, then this should be postponed until cooperation can be assured and it is safe for the clinician and patient.

Physical Health Assessment

- In consultation with primary and specialist health providers, all children must have physical health assessments and appropriate screening and monitoring in accordance with this policy.
- For children accessing CAMHS community-based services, CAMHS clinicians will work with the family to coordinate the ongoing physical health care needs of the child when they are unable to engage with a primary or specialist health care provider.
- Physical health assessments can be accepted by CAMHS services in an alternate format, however they must have been completed by a medical practitioner.
- Physical health assessments must be completed by a medical practitioner (including the Psychiatrist / Registrar) however growth charts can be completed by allied health staff, and physiological observations (including BMI and waist circumference) by nursing staff. The medical practitioner must be notified of any abnormal findings and other signs of deterioration.
- Physical health assessments must include, but not be limited to:
 - A comprehensive physical examination and medical history, both personal and relevant family history, to provide a baseline assessment of physical health and to exclude any medical causes for the mental illness (MHA 2014, s. 241 and CPG. 1.9.2, 1.9.3).
 - Enquiry into lifestyle and psychosocial factors known to influence physical health, including nutritional risks.

- Identification of alcohol and other drug (AOD) use through the SSCD Initial Assessment and initiation of additional AOD screening and monitoring tools where issues are identified.
- [Growth charts](#) are to be completed for all children in line with NOCS and management plan updates (every three months at a minimum).
 - Growth can be measured by any clinician with oversight by the Consultant Psychiatrist / Registrar.
 - Refusal to be weighed must be documented in webPSOLIS.
- Identification of existing oral health issues and enquiry into access to regular dental health care (e.g. school dental services or private providers).
- Identification of potential health care risk related to the mental health treatment.
- Screening for movement disorders including for patients not on medication. See [CAMHS Psychotropic Medication Monitoring Adverse Physical Health Effects policy](#) which has links to The [Abnormal Involuntary Movement Scale](#) (AIMS) that may be used to assist with the monitoring of involuntary movement/akathisia if clinically appropriate
- Ongoing follow up and monitoring of any abnormal findings and for the emergence of physical health care needs, in collaboration with a child's primary or specialist healthcare provider where possible

Physical health assessment – Ward 5A and Nickoll Ward

- Physical health assessment upon admission to CAMHS Ward 5A and Nickoll Ward, includes voluntary, involuntary or MIA patients.
- An initial physical examination must occur as soon as possible after admission and documented immediately in the Physical Examination form on webPSOLIS. However if there are any issues which preclude this (ie. webPSOLIS downtime) it can be done within 12 hours of admission.(MHA s.241 (2), CPG 1.9.9) unless:
 - an examination was conducted as part of inpatient care immediately preceding admission; or,
 - an examination is not possible due to the child's mental state or refusal to consent. This must be documented and the examination conducted as soon as practicable (MHA s. 241(2), CPG 1.9.11); or,
 - in the case of inter-ward transfer where the attending Registrar has noted in the medical record that they consider that the child has a sufficiently recent physical health assessment.
- Within 8 hours a pressure risk assessment must be completed as per the [PCH Pressure Injury Prevention And Management Guideline](#), using the 'Pressure Injury Prevention and Management Plan, MR851.07 form

- After the initial physical examination, examinations must be completed at reasonable intervals (MHA s. 241(2)) as clinically indicated.
- See [Appendix 1](#), Standard inpatient examination room equipment, for the standards necessary to enable physical health care assessments and monitoring of patients in Ward 5A/Nickoll Ward.
- A personalised physical healthcare care plan, including a plan for observations, monitoring and recording must be developed by the treating medical practitioner if any issues are determined to be under examination.
 - The child's diagnosis, presence of comorbidities, treatment and protocol requirements must be considered in the physical care plan. Clinicians should refer to [CAHS Policy Recognising and Responding to Acute Deterioration](#).
- The [Paediatric Acute Recognition and Response Observation Tool \(PARROT\)](#) is the general observation chart for measuring and documenting vital signs on Ward 5A and Nickoll Ward. All Ward 5A/Nickoll Ward patients are to have clinical observations measured as outlined in this policy.
- For children where physical restraint is judged to be a likely occurrence; all predisposing risk factors for adverse physical health outcomes of physical restraints must be assessed.
 - These include cardiac conditions; obesity; use of alcohol and drugs; natural disease and medications. See [CAMHS Physical Restraint Policy](#).
- For inpatients, pressure injury risk using Physical Examination Pressure Risk and Wound Assessment (MR510.03) and falls risk using Falls Risk Assessment Tool and Management Plan (MR851.04)

Physical health assessment in community-based teams / Outpatients

- A physical health assessment must be coordinated by a CAMHS clinician for all children after the child has been accepted for treatment (at partnership stage in Community CAMHS) and before the first clinical/multidisciplinary review at three months.
- When a physical examination and medical history has not been recently undertaken by a CAMHS clinician (including during admission to Ward 5A or Nickoll Ward) or another relevant health provider, a request for a physical health assessment should be sent to the child's General Practitioner for completion, either directly or via the family/carer.
 - Clinicians may use the [template provided](#), including the [CAMHS Physical Examination SSCD form](#) for communication with the child's GP.
 - Clinicians need to record in the medical record that a request for a physical health assessment was sent to the child's GP.
 - It is the responsibility of the Consultant Psychiatrist / Registrar to follow this up within 2 weeks if the form is not returned.

- Children receiving continuous care from a CAMHS team for more than 12 months should be scheduled for regular assessments at a minimum interval of three months. This quarterly timeframe serves as a baseline, with additional re-examinations conducted as clinically warranted.
- If no response is received from the GP either for the initial or yearly physical health assessment, then evidence of further attempts to communicate with the General Practitioner must be completed and filed in the medical record.
- If the child and family is unable or unwilling to engage with a GP/primary health care provider, a CAMHS Psychiatrist or delegate must provide the physical health assessment.
- See [Appendix 1](#), Standard Outpatient examination room equipment, for the standards necessary to enable physical health care assessments and monitoring of patients in all outpatient areas.
- See [Appendix 2](#) Physical Health – Community based teams/outpatients (excluding children under the Mental Health Act) Flowchart

Physical health assessment and monitoring for children prescribed psychotropic medication

- Clinicians should refer to [CAMHS Psychotropic Medication – Monitoring Adverse Physical Health Effects policy](#) and [CAMHS Psychotropic Medication Physical Health Monitoring Handbook](#) for information regarding;
 - requirements for baseline assessments and investigations prior to commencement of psychotropic medications; and
 - monitoring for the emergence of physical health care needs and side effects.

Physical health assessment and monitoring for young people with disordered eating

- **Inpatient**
 - Physical health assessment and monitoring must occur as outlined in the [PCH Eating Disorder: Medical Management for Compromised Nutritional Status guideline](#). A quick reference table is also available within the [pre-referral guidelines for the CAMHS Eating Disorders Service \(EDS\)](#).
 - At minimum:
 - Height must be taken on admission;
 - Weight must be taken twice weekly;
 - Bloods must be taken twice weekly; and
 - BMI as per: [Percentage of mBMI \(median BMI\) for age and gender](#)

- **Outpatient**

- Physical health assessment and monitoring is dependent on the young person's current percentage medium BMI. See reference table:
 - [Percentage of mBMI \(median BMI\) for age and gender](#)
- If the young person's current BMI is above the 85th percentile, weight must be taken monthly and counselling about lifestyle modifications be sensitively provided.
- If the young person's current BMI is below the 15th percentile, and there are concerns about intentional caloric restriction, consider requesting a consultation from the Eating Disorder Service.
- Staff must refer to the [pre-referral guidelines for the CAMHS Eating Disorders Service \(EDS\)](#). Staff must seek specialist advice from EDS where required.

Follow-up/discharge/transfer of care

- Taking into account confidentiality, clinicians must arrange adequate follow-up for identified physical health care needs as clinically indicated, in consultation with primary and specialist health care providers. This includes dental health care.
- Information regarding physical health care needs must be communicated appropriately at handover and discharge to support optimal follow up.
- Recovery plans, Care Transfer Summary or Notifications and Clinical Summaries (NaCS) must specifically address physical health care needs.

Documentation

- Consent and determination of capacity to consent must be documented in the medical record.
- All relevant information regarding a consumer's physical health care needs must be documented in the child's medical record.
- When the physical examination is completed by a CAMHS clinician, it must be documented using the [Physical Examination form](#) in webPSOLIS.
- Documentation must include use of designated forms, monitoring tools and communication templates, as per CAMHS [Psychotropic Medication – Monitoring Adverse Physical Health Effects policy](#) and [CAMHS Psychotropic Medication Physical Health Monitoring Handbook](#).
- Arrangements made in collaboration with a child's family for monitoring the child's physical health, are to be documented in the child's medical record.
- The advice to children and families regarding yearly physical health assessments and screening must be documented in the child's medical record, including that advice has been received and understood.

- Evidence of communication with primary and specialist care providers (e.g. letters, emails, progress notes detailing phone conversations) must be present in the clinical record and clearly identifiable.
- Clinicians can use the [Template GP letter](#) to communicate results, or ensure that all of the information within the template is communicated in writing to the ongoing health care provider.

Notifiable event – provision of urgent non-psychiatric treatment

- [Form 9B Report to the Chief Psychiatrist about Provision of Urgent non-Psychiatric Treatment](#) must be completed following emergency non-psychiatric treatment for involuntary inpatients as this is a notifiable event.

Compliance

- Compliance with this policy will be audited annually, using an approved audit tool. Measures include:
 - Completed [CAMHS Physical Examination SSCD form](#) in the medical record.
 - Evidence of communication with GP regarding Physical Health Assessment.
 - Incorporation of physical health care components in recovery plans.
 - Evidence of appropriate treatment for abnormal physical health states (evidence from the progress notes).
 - Physical examination documented within 12 hours of admission to Ward 5A or Nickoll Ward.

Related CAHS internal policies, procedures and guidelines
CAMHS Consent Guideline
CAMHS Clinical Assessment Policy
CAMHS Psychotropic medication – monitoring adverse physical health effects policy
CAMHS Physical Restraint Policy.
PCH Paediatric Acute Recognition and Response Observation Tool (PARROT)
PCH Pressure Injury Prevention And Management Guideline
PCH Eating Disorder: Medical Management for Compromised Nutritional Status
CAHS Policy Recognising and Responding to Acute Deterioration

References and related external legislation, policies, and guidelines
<u>Chief Psychiatrist's Standards for Clinical Care (CPSCC)</u>
<u>Mental Health Act (WA) 2014</u>
<u>MP 0155/21 - State-wide Standardised Clinical Documentation for Mental Health Services</u>
<u>MP 0062/17 - Alcohol and Other Drug Withdrawal Management Policy</u>
<u>MP 0175/22 - Consent to Treatment Policy</u>
<u>MP 0171/22 - Recognising and Responding to Acute Deterioration Policy</u>
Useful resources (including related forms)
<u>Abnormal Involuntary Movement Scale</u>
<u>CAMHS Physical Examination SSCD form</u>
<u>CAMHS Psychotropic Medication Physical Health Monitoring Handbook</u>
<u>Letter to GP Template</u>
<u>Paediatric Acute Recognition and Response Observation Tool (PARROT)</u>
<u>Percentage of mBMI (median BMI) for age and gender</u>
<u>Pre-referral Guidelines – CAMHS Eating Disorders Service</u>
<u>WHO Child Growth Standards</u>

This document can be made available in alternative formats on request.

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Appendix 1: Standard Examination Room Equipment

Minimum room requirements - Inpatient

- Private, warm, well-lit area.
- Examination couch or bed suitable for conducting physical examinations.
- Supply of clean sheets or towels.
- Sink.

Minimum equipment requirements - Inpatient

- | | |
|------------------------|--------------------------------|
| • Stethoscope | • Auriscope and ophthalmoscope |
| • Sphygmomanometer | • Snellen chart |
| • Thermometer | • Penlight |
| • Tendon hammer | • Height measure |
| • Tuning fork (256 Hz) | • Disposable gloves |
| • Weighing scales | • Examination lubricant |
| • Tape measure | • Neurological testing pins |
| • Urinalysis sticks | • Oximeter |

Extended Physical examination and investigations - Inpatient

- | | |
|--|---|
| • Peakflow monitor | • Sharps bin |
| • Glucometer | • First Aid kit - for staff and patients |
| • Alcometer/breathalyser | • Small drug cupboard |
| • Pathology venipuncture and associated collection equipment (only required if not outsourced to a Pathology Centre) | • Resuscitation trolley |
| • Pathology specimen containers (only required if not outsourced to a Pathology Centre) | • Access to an electrocardiograph machine |
| | • Body Mass Index wheel |

Minimum room requirements – Outpatient

- Private, warm, well-lit area.
- Examination couch/bed/chair suitable for conducting physical examinations.

- Sink.
- Documentation for recording examination findings.

Minimum equipment requirements – Outpatient / Community - based

- Stethoscope
- Sphygmomanometer
- Scales
- Height chart
- BMI chart
- Tendon hammer
- Thermometer
- Auriscope and ophthalmoscope.

Minimum maintenance requirements

Minimum maintenance should include cleaning of the room and equipment, annual functional check of the equipment and replacement of damaged equipment.

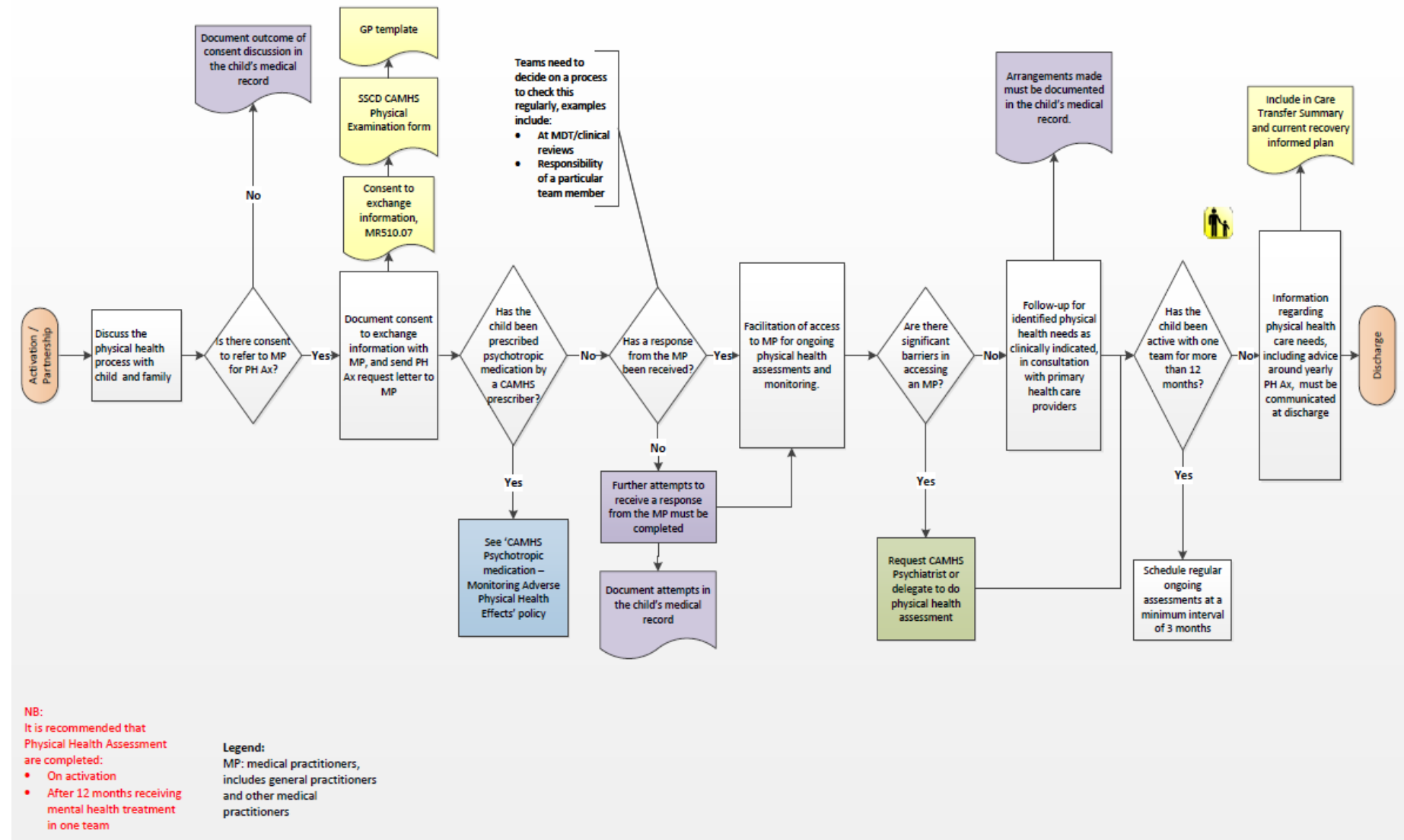
Minimum requirements for compliance

Yearly audits must be undertaken to monitor compliance with this policy. Service managers are responsible for ensuring this audit takes place. See [CAHS Asset Management Policy](#) for further details.

Managers must ensure that:

- staff have timely access to an examination room meeting the requirements described in this policy;
- the examination room is maintained. Including;
 - Cleaning of the room and equipment pre and post use.
 - Three monthly functional check of the equipment (note that Resuscitation trolleys require more frequent review, as per CAHS requirements).
 - Replacement of damaged equipment and expired items

Appendix 2: Physical Health Assessment Flowchart – Community



Appendix 3: Physical Health Assessment Flowchart – Ward 5A and Nickoll Ward

